

Selected Secondary Headache Disorders

Presentation:

- Age >50, new headache, **jaw claudication**, visual changes, tender temporal artery, high ESR/CRP: Giant Cell Arteritis (GCA).
- Young, obese female, headache, **pulsatile tinnitus**, vision changes, **papilledema** on exam: Idiopathic Intracranial Hypertension (IIH).
- Headache, confusion in winter, multiple family members affected: Carbon Monoxide poisoning.: Carbon Monoxide Poisoning.

Core Principles for Secondary Headaches:

1. If GCA is suspected with visual symptoms: Start high-dose steroids **IMMEDIATELY**. Do not wait for temporal artery biopsy.
2. If IIH is suspected: Obtain brain imaging (MRI/MRV), then confirm with Lumbar Puncture showing elevated opening pressure. Treat with Acetazolamide and weight loss.
3. If you suspect CO poisoning: Check carboxyhemoglobin level. Treat with 100% Oxygen.

Traumatic Brain Injury / Concussion

Presentation:

- Head trauma with confusion and amnesia, without focal deficits: Concussion.
- Brief loss of consciousness followed by a "**lucid interval**" and rapid decline. CT shows **lenticiform (lens-shaped)** hematoma: Epidural Hematoma (EDH).
- Gradual onset of headache/confusion in an elderly/alcoholic patient after a fall. CT shows **crescent-shaped** hematoma: Subdural Hematoma (SDH).

Core Principles for TBI:

1. Use clinical decision rules (e.g., Canadian CT Head Rule) to guide imaging. Image if: GCS <15, age >65, vomiting, or dangerous mechanism.
2. Management of concussion is cognitive and physical rest with gradual return to activity.
3. **Post-concussive syndrome** (headache, dizziness, cognitive slowing) is common and managed with reassurance and symptomatic treatment.

Epilepsy

- Recurrent, unprovoked seizures. Differentiate Focal vs. Generalized onset to guide therapy.